

Background

- Disease modifying therapies (DMTs) reduce relapse and disability progression rates among patients with Multiple Sclerosis (pwMS).
- Patient reported outcomes (PRO) can measure health-related quality of life, disease symptoms and burden, and health behaviors.
- Most DMTs are specialty medications requiring an extensive patient management program with close and consistent follow up.
- Adherence can be challenging in this patient population due to the high economic and social burden of MS.
- In May 2023, the integrated HSSP migrated to a new disease specific patient management platform.
- PwMS were enrolled in the MS disease program, with specific activities and disease management support tools.

Objective

- To evaluate the patient reported outcomes, and adherence among pwMS on self-administered DMTs.

Methods

- This is a retrospective chart review of completed activities by pharmacy staff between October 1, 2023, to September 30, 2024, for pwMS being treated with DMTs
- PwMS who filled their self-administered DMT (oral or injectable) at the HSSP and had at least one clinical assessment were included in the analysis
- The primary outcome included evaluating adherence of DMTs
- The total number of clinical assessments and PROs were evaluated as secondary outcomes

Results

Table 1: Demographic data

Total patients	218
Female	78%
Male	22%
Average age (yrs)	57

Table 2: DMT Drug Utilization in pwMS

Ofatumumab	22%
Dimethyl fumarate	21%
Interferons	12%
Glatiramer acetate	12%

Figure 1: Adherence to DMTs

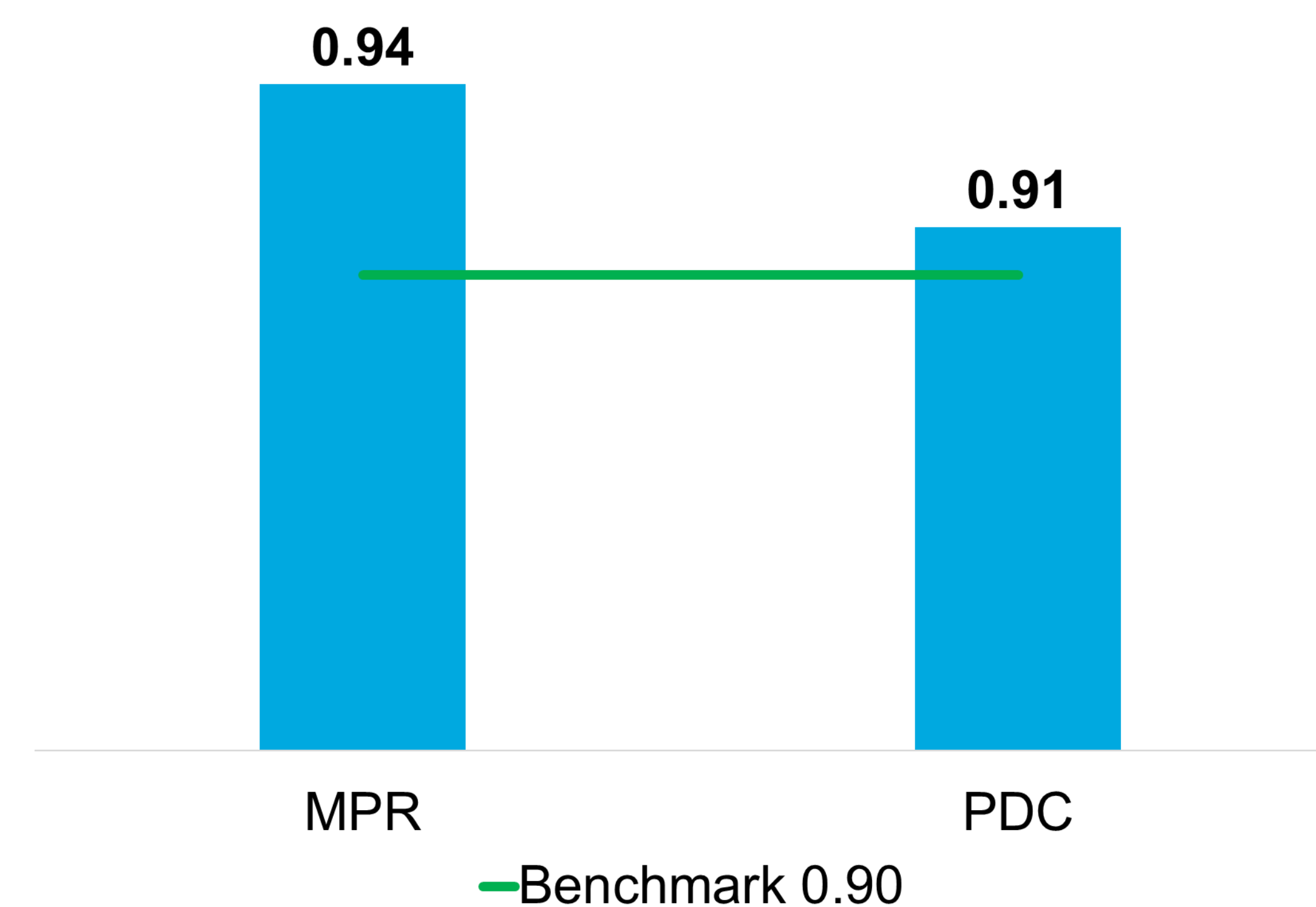


Figure 2: Clinical Services

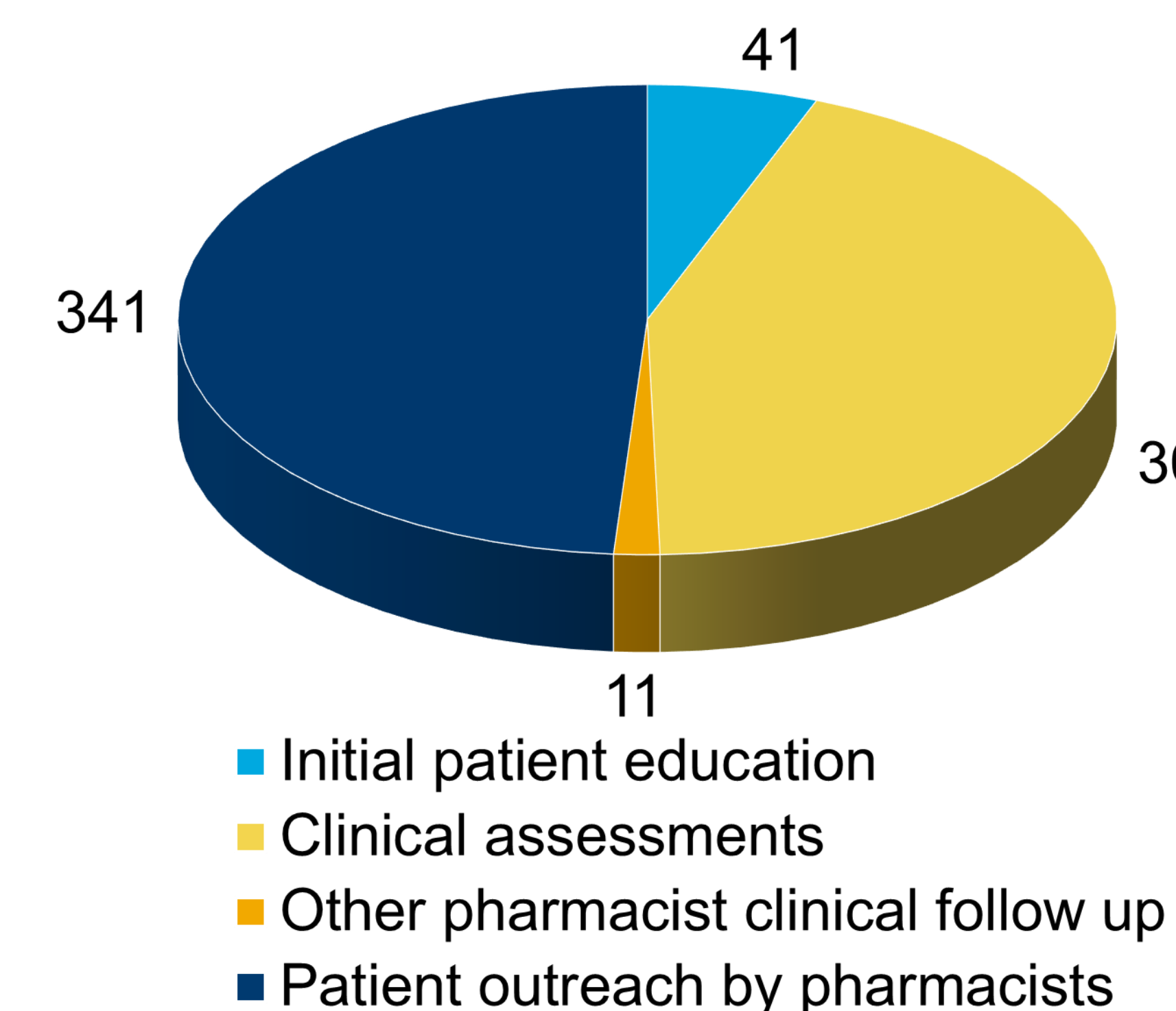


Figure 3: Patient reported zero missed doses

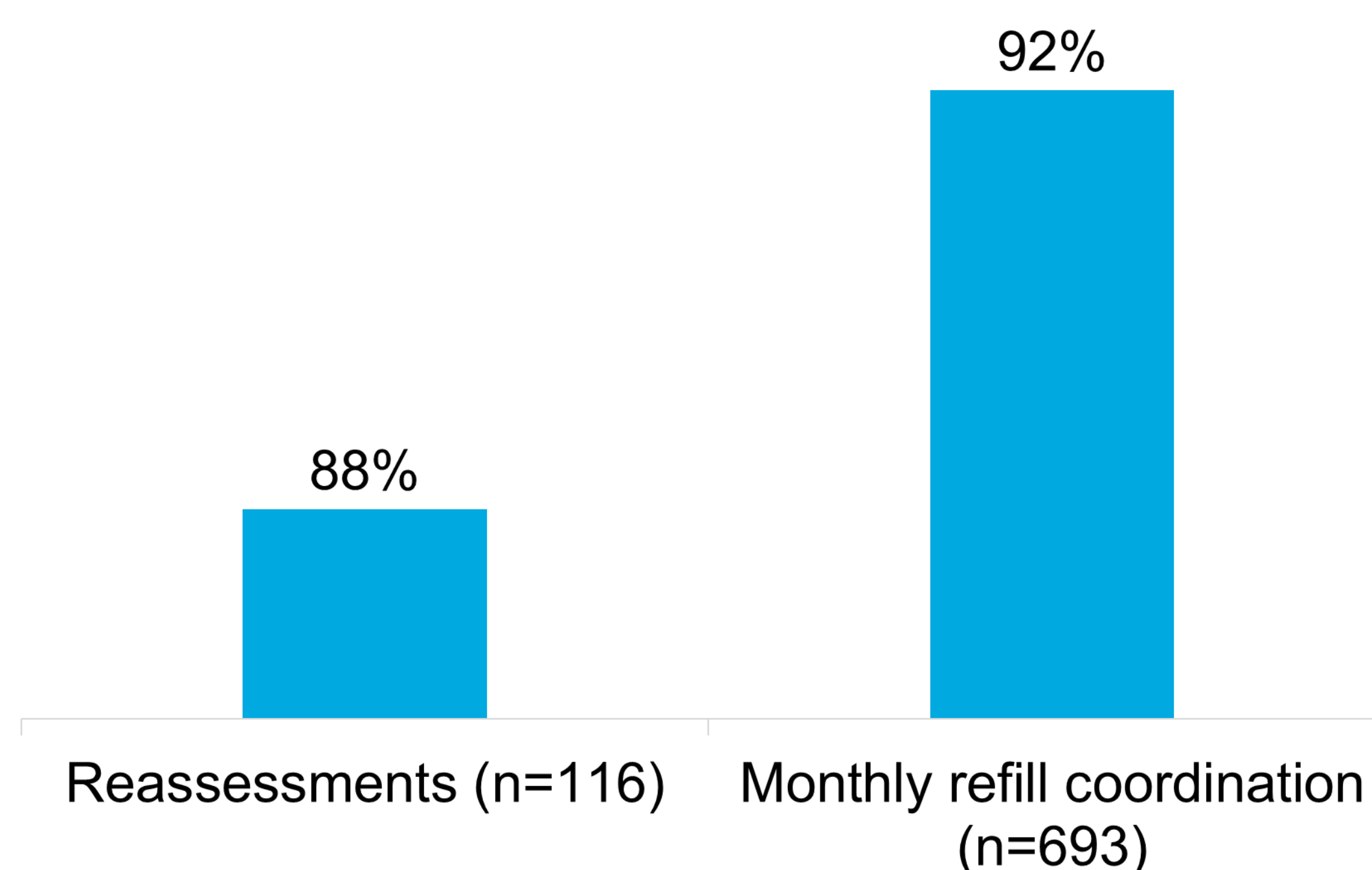
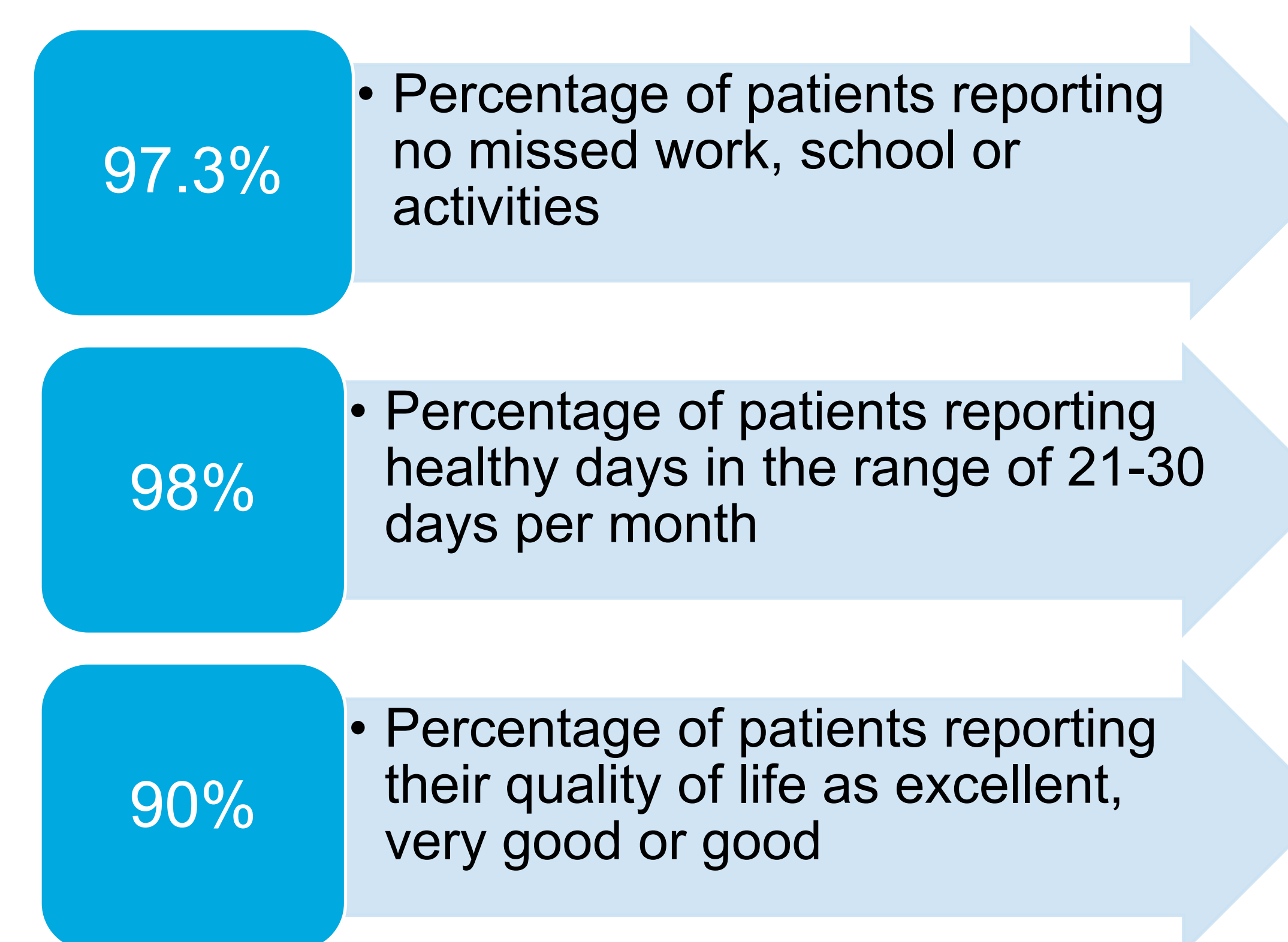


Figure 4: Patient reported outcomes



Discussion

- Literature reviews have cited varying MPR and PDC for DMTs. A systematic review reported MPR/PDC of 83.3%/ 76.5% for oral DMTs. Other studies have reported MPR of 69% for self injectable DMTs and 68% for oral DMTs.
- The MPR/PDC remains high at the integrated HSSP exceeding the benchmark goal for both measures.
- Non-adherent patients are more likely to discontinue treatment and report missed doses. Patients at the HSSP reporting zero missed doses varied from 88% to 92% during reassessments and monthly refill outreach.
- The close monitoring and follow up by the clinical team as evidenced by clinical services resulted in favorable patient reported outcomes.
- Studies have reported work difficulties, stigma, and poor quality of life even in a population with low physical disability among pwMS.
- The integrated specialty pharmacy model enables pharmacists to provide targeted interventions, enhances medication access, and improves coordination of care all of which contribute to better patient outcomes.

Limitations

- This review includes pwMS of all types, patients may experience higher levels of impairment and disability based on age of diagnosis, number of relapses etc.
- Varying MPR/PDC reported in literature can be due to different methods of calculation.
- PROs are based on just patients reached for reassessments.

Conclusion

Specialty clinical pharmacists are uniquely positioned to improve patient outcomes and adherence among pwMS.

References

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